

PROPOSED
HOUSE OF REPRESENTATIVES AMENDMENTS TO S.B. 1399
(Reference to Senate engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.

~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

~~<<Green strikeout in carets>>~~ indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 36, chapter 29, article 1, Arizona Revised
3 Statutes, is amended by adding section 36-2904.01, to read:

4 36-2904.01. Prepaid capitated contracts; expenditures; annual
5 report

6 ON OR BEFORE DECEMBER 1 OF EACH YEAR, THE ADMINISTRATION SHALL
7 ANNUALLY REPORT TO THE JOINT LEGISLATIVE BUDGET COMMITTEE, THE GOVERNOR,
8 THE PRESIDENT OF THE SENATE, THE SPEAKER OF THE HOUSE OF REPRESENTATIVES
9 AND THE MEMBERS OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND
10 HUMAN SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, IN AGGREGATE FOR
11 ALL NONGOVERNMENT ENTITIES THAT HAVE A PREPAID CAPITATED CONTRACT WITH THE
12 ADMINISTRATION, THE PERCENTAGE OF MONIES IN THE PREVIOUS CONTRACT YEAR:

13 1. THAT WAS SPENT ON DIRECT PAYMENTS TO PROVIDERS FOR SERVICES.

14 2. THAT WAS SPENT ON ADMINISTRATIVE COSTS FOR EACH OF THE
15 FOLLOWING:

16 (a) HEALTH CARE QUALITY IMPROVEMENT ADMINISTRATIVE ACTIVITIES.

17 (b) PROGRAM INTEGRITY, INCLUDING FRAUD, WASTE AND ABUSE PREVENTION
18 EXPENSES.

19 (c) TAXES, LICENSES AND REGULATORY FEES.

20 (d) COMMUNITY REINVESTMENT.

21 (e) ALL OTHER ADMINISTRATIVE ACTIVITIES.

1 <<Sec. 2. Title 36, chapter 29, article 1, Arizona Revised
2 Statutes, is amended by adding section 36-2930.07, to read:

3 36-2930.07. Managed care organizations; high-volume service
4 providers; termination without cause; written
5 notice; determination of network adequacy;
6 definitions

7 [A. BEFORE A MANAGED CARE ORGANIZATION MAY TERMINATE A HIGH-VOLUME
8 SERVICE PROVIDER'S CONTRACT WITHOUT CAUSE, THE MANAGED CARE ORGANIZATION
9 SHALL SUBMIT WRITTEN NOTICE TO THE ADMINISTRATION AT LEAST NINETY DAYS
10 BEFORE THE PROPOSED EFFECTIVE DATE OF THE TERMINATION.

11 B. THE NOTICE TO THE ADMINISTRATION PURSUANT TO SUBSECTION A OF
12 THIS SECTION SHALL INCLUDE ALL OF THE FOLLOWING:

13 1. DOCUMENTATION SHOWING THAT THE SERVICE PROVIDER IS A HIGH-VOLUME
14 SERVICE PROVIDER, INCLUDING THE SPECIFIC DATA SOURCES, CALCULATION
15 METHODOLOGY AND METRICS USED.

16 2. A NETWORK ADEQUACY STUDY PERFORMED BY THE MANAGED CARE
17 ORGANIZATION THAT EVALUATES AND DOCUMENTS, AT A MINIMUM:

18 (a) CURRENT AND PROJECTED POSTTERMINATION SERVICE
19 PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE AND GEOGRAPHIC
20 SERVICE AREA.

21 (b) CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE AFFECTED
22 SERVICES.

23 (c) THE PATIENT VOLUME AND GEOGRAPHIC DISTRIBUTION OF THE AFFECTED
24 SERVICES.

25 (d) THE IMPACT ON MEMBERS WHO ARE RECEIVING BEHAVIORAL HEALTH
26 SERVICES ASSOCIATED WITH THE MEMBER'S DISABILITY.

27 (e) THE CUMULATIVE EFFECT OF ALL PENDING OR RECENTLY COMPLETED
28 WITHOUT-CAUSE TERMINATIONS OF HIGH-VOLUME SERVICE PROVIDERS BY THE MANAGED
29 CARE ORGANIZATION.

30 (f) ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
31 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.

32 3. THE MANAGED CARE ORGANIZATION'S PRELIMINARY ASSESSMENT OF THE
33 IMPACT OF THE TERMINATION ON NETWORK ADEQUACY.

34 4. ANY MITIGATION MEASURES THE MANAGED CARE ORGANIZATION IS
35 PROPOSING.

36 C. IF THERE IS A DISCREPANCY BETWEEN THE MANAGED CARE ORGANIZATION
37 AND THE SERVICE PROVIDER OF WHETHER THE SERVICE PROVIDER IS A HIGH-VOLUME
38 SERVICE PROVIDER, THE MANAGED CARE ORGANIZATION SHALL NOTIFY THE
39 ADMINISTRATION AND PROVIDE DOCUMENTATION SUPPORTING THE MANAGED CARE
40 ORGANIZATION'S DECISION NOT TO FILE THE WRITTEN NOTICE PURSUANT TO
41 SUBSECTION A OF THIS SECTION. THE ADMINISTRATION SHALL REVIEW THE
42 DOCUMENTATION AND DECIDE WHETHER THE MANAGED CARE ORGANIZATION IS REQUIRED
43 TO FILE WRITTEN NOTICE PURSUANT TO SUBSECTION A OF THIS SECTION. THE
44 ADMINISTRATION SHALL NOTIFY THE SERVICE PROVIDER OF THE DECISION.

45 D. A MANAGED CARE ORGANIZATION MAY NOT TERMINATE A HIGH-VOLUME
46 SERVICE PROVIDER WITHOUT CAUSE UNTIL THE ADMINISTRATION HAS REVIEWED THE
47 MANAGED CARE ORGANIZATION'S NETWORK ADEQUACY STUDY AND HAS PROVIDED

1 WRITTEN CONFIRMATION THAT APPLICABLE NETWORK ADEQUACY STANDARDS WILL
2 CONTINUE TO BE MET AFTER THE TERMINATION OF THE HIGH-VOLUME SERVICE
3 PROVIDER. THE ADMINISTRATION SHALL COMPLETE ITS REVIEW OF THE NETWORK
4 ADEQUACY STUDY WITHIN TEN BUSINESS DAYS AFTER RECEIVING THE NOTICE
5 PURSUANT TO SUBSECTION A OF THIS SECTION.

6 E. IF THE ADMINISTRATION AGREES WITH THE MANAGED CARE
7 ORGANIZATION'S DECISION BASED ON THE FINDINGS PROVIDED, THE ADMINISTRATION
8 SHALL BOTH:

9 1. POST THE MANAGED CARE ORGANIZATION'S COMPLETE NETWORK ADEQUACY
10 STUDY, INCLUDING METHODOLOGY, DATA SOURCES, METRICS AND FINDINGS, AND THE
11 ADMINISTRATION'S DETERMINATION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND
12 SEND A COPY TO THE CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES
13 HEALTH AND HUMAN SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND
14 THE GOVERNOR'S OFFICE.

15 2. PROVIDE WRITTEN NOTICE OF THE ADMINISTRATION'S DETERMINATION TO
16 THE MANAGED CARE ORGANIZATION.

17 F. IF THE ADMINISTRATION DETERMINES THAT NETWORK ADEQUACY STANDARDS
18 WOULD NOT BE MET, THE MANAGED CARE ORGANIZATION MAY NOT PROCEED WITH THE
19 HIGH-VOLUME SERVICE PROVIDER'S TERMINATION UNLESS THE MANAGED CARE
20 ORGANIZATION DEMONSTRATES TO THE ADMINISTRATION'S SATISFACTION THAT
21 NETWORK ADEQUACY STANDARDS WILL BE MET.

22 G. IF A MANAGED CARE ORGANIZATION DECLINES TO CONTRACT WITH A
23 SERVICE PROVIDER OR POTENTIAL SERVICE PROVIDER DUE TO A DETERMINATION OF
24 NETWORK ADEQUACY, THE MANAGED CARE ORGANIZATION SHALL COMPLETE AND SEND TO
25 THE ADMINISTRATION A NETWORK ADEQUACY STUDY, INCLUDING THE SPECIFIC DATA
26 SOURCES, CALCULATION METHODOLOGY AND METRICS USED TO SUPPORT THE DENIAL
27 DECISION. THE STUDY MUST INCLUDE, AT A MINIMUM:

28 1. SERVICE PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE.

29 2. CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE SERVICES THAT
30 THE APPLICANT WAS TO PROVIDE.

31 3. ALL PENDING OR RECENTLY COMPLETED TERMINATIONS OF HIGH-VOLUME
32 SERVICE PROVIDERS WITHOUT CAUSE IN THE GEOGRAPHIC AREA THAT THE APPLICANT
33 WOULD HAVE SERVED.

34 4. ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
35 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.

36 H. SUBSECTION G OF THIS SECTION APPLIES ONLY TO HIGH-VOLUME SERVICE
37 PROVIDERS.

38 I. THE ADMINISTRATION SHALL POST THE MANAGED CARE ORGANIZATION'S
39 COMPLETE NETWORK ADEQUACY STUDY SUBMITTED PURSUANT TO SUBSECTION G OF THIS
40 SECTION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND SEND A COPY TO THE
41 CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN
42 SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND THE GOVERNOR'S
43 OFFICE.

1 J. FOR THE PURPOSES OF THIS SECTION:

2 1. "HIGH-VOLUME SERVICE PROVIDER" MEANS A SERVICE PROVIDER THAT
3 MEETS EITHER OF THE FOLLOWING:

4 (a) DELIVERED AT LEAST TEN PERCENT OF ANY SPECIFIC SERVICE FOR A
5 MANAGED CARE ORGANIZATION IN THE PRECEDING STATE FISCAL YEAR.

6 (b) EMPLOYS MORE THAN TEN PERCENT OF THE ACTIVELY LICENSED
7 BEHAVIORAL HEALTH PROVIDERS IN THIS STATE.

8 2. "MANAGED CARE ORGANIZATION" MEANS A CONTRACTOR THAT HAS A
9 PREPAID CAPITATED CONTRACT WITH THE ADMINISTRATION OR A REGIONAL
10 BEHAVIORAL HEALTH AUTHORITY.

11 3. "SERVICE PROVIDER" MEANS AN ORGANIZATION OR MENTAL HEALTH
12 PROFESSIONAL THAT MEETS THE CRITERIA ESTABLISHED BY THE ADMINISTRATION AND
13 THAT HAS A CONTRACT WITH THE ADMINISTRATION OR A REGIONAL BEHAVIORAL
14 HEALTH AUTHORITY.]>>

15 <<Sec. 3. Title 36, chapter 29, article 2, Arizona Revised
16 Statutes, is amended by adding section 36-2961, to read:

17 36-2961. Managed care organizations; high-volume service
18 providers; termination without cause; written
19 notice; determination of network adequacy;
20 definitions

21 [A. BEFORE A MANAGED CARE ORGANIZATION MAY TERMINATE A HIGH-VOLUME
22 SERVICE PROVIDER'S CONTRACT WITHOUT CAUSE, THE MANAGED CARE ORGANIZATION
23 SHALL SUBMIT WRITTEN NOTICE TO THE ADMINISTRATION AT LEAST NINETY DAYS
24 BEFORE THE PROPOSED EFFECTIVE DATE OF THE TERMINATION.

25 B. THE NOTICE TO THE ADMINISTRATION PURSUANT TO SUBSECTION A OF
26 THIS SECTION SHALL INCLUDE ALL OF THE FOLLOWING:

27 1. DOCUMENTATION SHOWING THAT THE SERVICE PROVIDER IS A HIGH-VOLUME
28 SERVICE PROVIDER, INCLUDING THE SPECIFIC DATA SOURCES, CALCULATION
29 METHODOLOGY AND METRICS USED.

30 2. A NETWORK ADEQUACY STUDY PERFORMED BY THE MANAGED CARE
31 ORGANIZATION THAT EVALUATES AND DOCUMENTS, AT A MINIMUM:

32 (a) CURRENT AND PROJECTED POSTTERMINATION SERVICE
33 PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE AND GEOGRAPHIC
34 SERVICE AREA.

35 (b) CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE AFFECTED
36 SERVICES.

37 (c) THE PATIENT VOLUME AND GEOGRAPHIC DISTRIBUTION OF THE AFFECTED
38 SERVICES.

39 (d) THE IMPACT ON MEMBERS WHO ARE RECEIVING BEHAVIORAL HEALTH
40 SERVICES ASSOCIATED WITH THE MEMBER'S DISABILITY.

41 (e) THE CUMULATIVE EFFECT OF ALL PENDING OR RECENTLY COMPLETED
42 WITHOUT-CAUSE TERMINATIONS OF HIGH-VOLUME SERVICE PROVIDERS BY THE MANAGED
43 CARE ORGANIZATION.

44 (f) ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
45 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.

46 3. THE MANAGED CARE ORGANIZATION'S PRELIMINARY ASSESSMENT OF THE
47 IMPACT OF THE TERMINATION ON NETWORK ADEQUACY.

1 4. ANY MITIGATION MEASURES THE MANAGED CARE ORGANIZATION IS
2 PROPOSING.

3 C. IF THERE IS A DISCREPANCY BETWEEN THE MANAGED CARE ORGANIZATION
4 AND THE SERVICE PROVIDER OF WHETHER THE SERVICE PROVIDER IS A HIGH-VOLUME
5 SERVICE PROVIDER, THE MANAGED CARE ORGANIZATION SHALL NOTIFY THE
6 ADMINISTRATION AND PROVIDE DOCUMENTATION SUPPORTING THE MANAGED CARE
7 ORGANIZATION'S DECISION NOT TO FILE THE WRITTEN NOTICE PURSUANT TO
8 SUBSECTION A OF THIS SECTION. THE ADMINISTRATION SHALL REVIEW THE
9 DOCUMENTATION AND DECIDE WHETHER THE MANAGED CARE ORGANIZATION IS REQUIRED
10 TO FILE WRITTEN NOTICE PURSUANT TO SUBSECTION A OF THIS SECTION. THE
11 ADMINISTRATION SHALL NOTIFY THE SERVICE PROVIDER OF THE DECISION.

12 D. A MANAGED CARE ORGANIZATION MAY NOT TERMINATE A HIGH-VOLUME
13 SERVICE PROVIDER WITHOUT CAUSE UNTIL THE ADMINISTRATION HAS REVIEWED THE
14 MANAGED CARE ORGANIZATION'S NETWORK ADEQUACY STUDY AND HAS PROVIDED
15 WRITTEN CONFIRMATION THAT APPLICABLE NETWORK ADEQUACY STANDARDS WILL
16 CONTINUE TO BE MET AFTER THE TERMINATION OF THE HIGH-VOLUME SERVICE
17 PROVIDER. THE ADMINISTRATION SHALL COMPLETE ITS REVIEW OF THE NETWORK
18 ADEQUACY STUDY WITHIN TEN BUSINESS DAYS AFTER RECEIVING THE NOTICE
19 PURSUANT TO SUBSECTION A OF THIS SECTION.

20 E. IF THE ADMINISTRATION AGREES WITH THE MANAGED CARE
21 ORGANIZATION'S DECISION BASED ON THE FINDINGS PROVIDED, THE ADMINISTRATION
22 SHALL BOTH:

23 1. POST THE MANAGED CARE ORGANIZATION'S COMPLETE NETWORK ADEQUACY
24 STUDY, INCLUDING METHODOLOGY, DATA SOURCES, METRICS AND FINDINGS, AND THE
25 ADMINISTRATION'S DETERMINATION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND
26 SEND A COPY TO THE CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES
27 HEALTH AND HUMAN SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND
28 THE GOVERNOR'S OFFICE.

29 2. PROVIDE WRITTEN NOTICE OF THE ADMINISTRATION'S DETERMINATION TO
30 THE MANAGED CARE ORGANIZATION.

31 F. IF THE ADMINISTRATION DETERMINES THAT NETWORK ADEQUACY STANDARDS
32 WOULD NOT BE MET, THE MANAGED CARE ORGANIZATION MAY NOT PROCEED WITH THE
33 HIGH-VOLUME SERVICE PROVIDER'S TERMINATION UNLESS THE MANAGED CARE
34 ORGANIZATION DEMONSTRATES TO THE ADMINISTRATION'S SATISFACTION THAT
35 NETWORK ADEQUACY STANDARDS WILL BE MET.

36 G. IF A MANAGED CARE ORGANIZATION DECLINES TO CONTRACT WITH A
37 SERVICE PROVIDER OR POTENTIAL SERVICE PROVIDER DUE TO A DETERMINATION OF
38 NETWORK ADEQUACY, THE MANAGED CARE ORGANIZATION SHALL COMPLETE AND SEND TO
39 THE ADMINISTRATION A NETWORK ADEQUACY STUDY, INCLUDING THE SPECIFIC DATA
40 SOURCES, CALCULATION METHODOLOGY AND METRICS USED TO SUPPORT THE DENIAL
41 DECISION. THE STUDY MUST INCLUDE, AT A MINIMUM:

42 1. SERVICE PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE.

43 2. CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE SERVICES THAT
44 THE APPLICANT WAS TO PROVIDE.

45 3. ALL PENDING OR RECENTLY COMPLETED TERMINATIONS OF HIGH-VOLUME
46 SERVICE PROVIDERS WITHOUT CAUSE IN THE GEOGRAPHIC AREA THAT THE APPLICANT
47 WOULD HAVE SERVED.

1 4. ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
2 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.

3 H. SUBSECTION G OF THIS SECTION APPLIES ONLY TO HIGH-VOLUME SERVICE
4 PROVIDERS.

5 I. THE ADMINISTRATION SHALL POST THE MANAGED CARE ORGANIZATION'S
6 COMPLETE NETWORK ADEQUACY STUDY SUBMITTED PURSUANT TO SUBSECTION G OF THIS
7 SECTION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND SEND A COPY TO THE
8 CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN
9 SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND THE GOVERNOR'S
10 OFFICE.

11 J. FOR THE PURPOSES OF THIS SECTION:

12 1. "HIGH-VOLUME SERVICE PROVIDER" MEANS A SERVICE PROVIDER THAT
13 MEETS EITHER OF THE FOLLOWING:

14 (a) DELIVERED AT LEAST TEN PERCENT OF ANY SPECIFIC SERVICE FOR A
15 MANAGED CARE ORGANIZATION IN THE PRECEDING STATE FISCAL YEAR.

16 (b) EMPLOYS MORE THAN TEN PERCENT OF THE ACTIVELY LICENSED
17 BEHAVIORAL HEALTH PROVIDERS IN THIS STATE.

18 2. "MANAGED CARE ORGANIZATION" MEANS A CONTRACTOR THAT HAS A
19 PREPAID CAPITATED CONTRACT WITH THE ADMINISTRATION OR A REGIONAL
20 BEHAVIORAL HEALTH AUTHORITY.

21 3. "SERVICE PROVIDER" MEANS AN ORGANIZATION OR MENTAL HEALTH
22 PROFESSIONAL THAT MEETS THE CRITERIA ESTABLISHED BY THE ADMINISTRATION AND
23 THAT HAS A CONTRACT WITH THE ADMINISTRATION OR A REGIONAL BEHAVIORAL
24 HEALTH AUTHORITY.]>>

25 <<Sec. 4. Title 36, chapter 34, article 1, Arizona Revised
26 Statutes, is amended by adding section 36-3414, to read:

27 36-3414. Managed care organizations; high-volume service
28 providers; termination without cause; written
29 notice; determination of network adequacy;
30 definitions

31 [A. BEFORE A MANAGED CARE ORGANIZATION MAY TERMINATE A HIGH-VOLUME
32 SERVICE PROVIDER'S CONTRACT WITHOUT CAUSE, THE MANAGED CARE ORGANIZATION
33 SHALL SUBMIT WRITTEN NOTICE TO THE ADMINISTRATION AT LEAST NINETY DAYS
34 BEFORE THE PROPOSED EFFECTIVE DATE OF THE TERMINATION.

35 B. THE NOTICE TO THE ADMINISTRATION PURSUANT TO SUBSECTION A OF
36 THIS SECTION SHALL INCLUDE ALL OF THE FOLLOWING:

37 1. DOCUMENTATION SHOWING THAT THE SERVICE PROVIDER IS A HIGH-VOLUME
38 SERVICE PROVIDER, INCLUDING THE SPECIFIC DATA SOURCES, CALCULATION
39 METHODOLOGY AND METRICS USED.

40 2. A NETWORK ADEQUACY STUDY PERFORMED BY THE MANAGED CARE
41 ORGANIZATION THAT EVALUATES AND DOCUMENTS, AT A MINIMUM:

42 (a) CURRENT AND PROJECTED POSTTERMINATION SERVICE
43 PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE AND GEOGRAPHIC
44 SERVICE AREA.

45 (b) CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE AFFECTED
46 SERVICES.

1 (c) THE PATIENT VOLUME AND GEOGRAPHIC DISTRIBUTION OF THE AFFECTED
2 SERVICES.

3 (d) THE IMPACT ON MEMBERS WHO ARE RECEIVING BEHAVIORAL HEALTH
4 SERVICES ASSOCIATED WITH THE MEMBER'S DISABILITY.

5 (e) THE CUMULATIVE EFFECT OF ALL PENDING OR RECENTLY COMPLETED
6 WITHOUT-CAUSE TERMINATIONS OF HIGH-VOLUME SERVICE PROVIDERS BY THE MANAGED
7 CARE ORGANIZATION.

8 (f) ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
9 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.

10 3. THE MANAGED CARE ORGANIZATION'S PRELIMINARY ASSESSMENT OF THE
11 IMPACT OF THE TERMINATION ON NETWORK ADEQUACY.

12 4. ANY MITIGATION MEASURES THE MANAGED CARE ORGANIZATION IS
13 PROPOSING.

14 C. IF THERE IS A DISCREPANCY BETWEEN THE MANAGED CARE ORGANIZATION
15 AND THE SERVICE PROVIDER OF WHETHER THE SERVICE PROVIDER IS A HIGH-VOLUME
16 SERVICE PROVIDER, THE MANAGED CARE ORGANIZATION SHALL NOTIFY THE
17 ADMINISTRATION AND PROVIDE DOCUMENTATION SUPPORTING THE MANAGED CARE
18 ORGANIZATION'S DECISION NOT TO FILE THE WRITTEN NOTICE PURSUANT TO
19 SUBSECTION A OF THIS SECTION. THE ADMINISTRATION SHALL REVIEW THE
20 DOCUMENTATION AND DECIDE WHETHER THE MANAGED CARE ORGANIZATION IS REQUIRED
21 TO FILE WRITTEN NOTICE PURSUANT TO SUBSECTION A OF THIS SECTION. THE
22 ADMINISTRATION SHALL NOTIFY THE SERVICE PROVIDER OF THE DECISION.

23 D. A MANAGED CARE ORGANIZATION MAY NOT TERMINATE A HIGH-VOLUME
24 SERVICE PROVIDER WITHOUT CAUSE UNTIL THE ADMINISTRATION HAS REVIEWED THE
25 MANAGED CARE ORGANIZATION'S NETWORK ADEQUACY STUDY AND HAS PROVIDED
26 WRITTEN CONFIRMATION THAT APPLICABLE NETWORK ADEQUACY STANDARDS WILL
27 CONTINUE TO BE MET AFTER THE TERMINATION OF THE HIGH-VOLUME SERVICE
28 PROVIDER. THE ADMINISTRATION SHALL COMPLETE ITS REVIEW OF THE NETWORK
29 ADEQUACY STUDY WITHIN TEN BUSINESS DAYS AFTER RECEIVING THE NOTICE
30 PURSUANT TO SUBSECTION A OF THIS SECTION.

31 E. IF THE ADMINISTRATION AGREES WITH THE MANAGED CARE
32 ORGANIZATION'S DECISION BASED ON THE FINDINGS PROVIDED, THE ADMINISTRATION
33 SHALL BOTH:

34 1. POST THE MANAGED CARE ORGANIZATION'S COMPLETE NETWORK ADEQUACY
35 STUDY, INCLUDING METHODOLOGY, DATA SOURCES, METRICS AND FINDINGS, AND THE
36 ADMINISTRATION'S DETERMINATION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND
37 SEND A COPY TO THE CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES
38 HEALTH AND HUMAN SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND
39 THE GOVERNOR'S OFFICE.

40 2. PROVIDE WRITTEN NOTICE OF THE ADMINISTRATION'S DETERMINATION TO
41 THE MANAGED CARE ORGANIZATION.

42 F. IF THE ADMINISTRATION DETERMINES THAT NETWORK ADEQUACY STANDARDS
43 WOULD NOT BE MET, THE MANAGED CARE ORGANIZATION MAY NOT PROCEED WITH THE
44 HIGH-VOLUME SERVICE PROVIDER'S TERMINATION UNLESS THE MANAGED CARE
45 ORGANIZATION DEMONSTRATES TO THE ADMINISTRATION'S SATISFACTION THAT
46 NETWORK ADEQUACY STANDARDS WILL BE MET.

1 G. IF A MANAGED CARE ORGANIZATION DECLINES TO CONTRACT WITH A
2 SERVICE PROVIDER OR POTENTIAL SERVICE PROVIDER DUE TO A DETERMINATION OF
3 NETWORK ADEQUACY, THE MANAGED CARE ORGANIZATION SHALL COMPLETE AND SEND TO
4 THE ADMINISTRATION A NETWORK ADEQUACY STUDY, INCLUDING THE SPECIFIC DATA
5 SOURCES, CALCULATION METHODOLOGY AND METRICS USED TO SUPPORT THE DENIAL
6 DECISION. THE STUDY MUST INCLUDE, AT A MINIMUM:
7 1. SERVICE PROVIDER-TO-ENROLLEE RATIOS BY SERVICE PROVIDER TYPE.
8 2. CURRENT APPOINTMENT WAIT TIME PERFORMANCE FOR THE SERVICES THAT
9 THE APPLICANT WAS TO PROVIDE.
10 3. ALL PENDING OR RECENTLY COMPLETED TERMINATIONS OF HIGH-VOLUME
11 SERVICE PROVIDERS WITHOUT CAUSE IN THE GEOGRAPHIC AREA THAT THE APPLICANT
12 WOULD HAVE SERVED.
13 4. ANY ADDITIONAL FACTORS THE MANAGED CARE ORGANIZATION OR THE
14 ADMINISTRATION IDENTIFIES AS RELEVANT TO NETWORK ADEQUACY.
15 H. SUBSECTION G OF THIS SECTION APPLIES ONLY TO HIGH-VOLUME SERVICE
16 PROVIDERS.
17 I. THE ADMINISTRATION SHALL POST THE MANAGED CARE ORGANIZATION'S
18 COMPLETE NETWORK ADEQUACY STUDY SUBMITTED PURSUANT TO SUBSECTION G OF THIS
19 SECTION ON THE ADMINISTRATION'S PUBLIC WEBSITE AND SEND A COPY TO THE
20 CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN
21 SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, AND THE GOVERNOR'S
22 OFFICE.
23 J. FOR THE PURPOSES OF THIS SECTION:
24 1. "HIGH-VOLUME SERVICE PROVIDER" MEANS A SERVICE PROVIDER THAT
25 MEETS EITHER OF THE FOLLOWING:
26 (a) DELIVERED AT LEAST TEN PERCENT OF ANY SPECIFIC SERVICE FOR A
27 MANAGED CARE ORGANIZATION IN THE PRECEDING STATE FISCAL YEAR.
28 (b) EMPLOYS MORE THAN TEN PERCENT OF THE ACTIVELY LICENSED
29 BEHAVIORAL HEALTH PROVIDERS IN THIS STATE.
30 2. "MANAGED CARE ORGANIZATION" MEANS A CONTRACTOR THAT HAS A
31 PREPAID CAPITATED CONTRACT WITH THE ADMINISTRATION OR A REGIONAL
32 BEHAVIORAL HEALTH AUTHORITY.]>>

33 Enroll and engross to conform
34 Amend title to conform

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